Family Experience

Objective

Riayah Plus is being built to simplify healthcare for patients and their families.

While the platform contains sophisticated clinical workflows, care planning, scheduling, compliance, documentation, AI assistance, and operational management, these processes should remain behind the scenes.

The Family App should always be simple, intuitive, and easy to navigate, regardless of the complexity of the underlying system.

The family experience should focus on receiving care, not managing healthcare operations.

Family App vs Staff Portal

Riayah Plus consists of two different experiences that serve different purposes.

Family App

The Family App is designed for patients and their families.

Its purpose is to make it easy to request care, communicate with the care team, and stay informed throughout the care journey.

Families should be able to:

- Request support
- Book consultations
- Join telehealth appointments
- View their care plan
- Track progress
- Receive reminders
- Upload documents
- Communicate with the care team
- Review quotations
- Pay invoices

The Family App should never expose internal operational or clinical workflows.

Staff Portal

The Staff Portal is where all operational and clinical management takes place.

This includes:

- Care Episodes
- Assessments
- Care Plans
- Task Management
- Scheduling
- Workforce Management
- Clinical Documentation
- Compliance
- Reporting
- AI-assisted clinical tools
- Audit logs

All operational complexity belongs here, not in the Family App.

Family Language

Medical and technical terminology used internally should be translated into simple language for families.

Internal System	Family App
Care Episode	Care Journey
Assessment	Consultation
Clinical Documentation	Visit Summary
Task	Upcoming Activity
Coordinator Notes	Care Team Notes
Goal	Care Goal

Outcome Report	Progress Summary

The Family App should always communicate in language that is easy to understand.

Family Onboarding

The first interaction with Riayah Plus should be quick and straightforward.

The objective is to help families request care, not complete lengthy medical forms.

The onboarding process should follow this sequence.

Step 1

How can we help you today?

Examples:

- Book a consultation
- Home care
- Nursing services
- Allied health
- Telehealth
- I'm not sure

Step 2

Who needs care?

- Myself
- My child
- My parent
- Another family member

Step 3

Basic Information

Only request:

- Full name
- Date of birth
- Mobile number
- Preferred language

Step 4

Tell us about the situation

Families should be able to:

- Describe their situation
- Upload medical reports
- Upload hospital discharge summaries
- Upload prescriptions

The system should avoid asking families to manually re-enter information that already exists in clinical documents.

Step 5

Book a Consultation

Allow families to choose:

- Clinic consultation
- Home visit
- Telehealth consultation

Available times should be presented automatically based on staff availability.

Step 6

Consent

Display only the required consent forms before submission.

Step 7

Confirmation

Display a simple confirmation message.

> Your consultation has been booked successfully.
>
> Our care team will review your information before your appointment.

The onboarding process ends here for the family.

Internal Processing

After submission, the platform should automatically perform all internal processes without requiring further action from the family.

This includes:

- Creating the Care Unit
- Creating the Patient Profile
- Creating the Intake Record
- Creating a Draft Care Episode
- Recording consent
- Saving uploaded documents
- Creating a review task for the coordinator
- Recording audit events

These processes should remain completely invisible to the family.

Family Dashboard

The Family Dashboard should focus only on information that is useful to the patient or family.

Recommended sections include:

- Home
- My Care Plan
- Appointments

- Telehealth
- Messages
- Documents
- Quotations & Payments
- Progress
- Book a Service

Internal workflow information should not appear in the Family App.

Examples include:

- Care Episode IDs
- Workflow statuses
- Internal approvals
- Clinical state transitions
- Technical identifiers

Care Journey

Families should see a simple visual representation of their care journey.

For example:

Consultation Booked

↓

Assessment Completed

↓

Care Plan Ready

↓

Services Started

↓

Progress Tracking

↓

Next Review

Internally, the system may manage many workflow states, but these should be presented as clear and meaningful milestones.

Booking Experience

Booking should be straightforward.

The family selects:

- Service
- Date
- Time

The platform automatically:

- Checks staff availability
- Validates service eligibility
- Matches the appropriate clinician
- Prevents scheduling conflicts
- Confirms the appointment

The family should not be required to choose a clinician unless a preferred clinician option is introduced in a future release.

Clinical Workflow

Once the consultation has been booked, all clinical and operational activities move to the Staff Portal.

Clinical staff and coordinators are responsible for:

- Reviewing submitted information
- Completing assessments
- Creating care plans
- Scheduling ongoing services
- Assigning staff
- Monitoring progress

- Managing reviews

The family only sees the outcome of these activities.

Support First

Not every family knows which service they need.

The Family App should always provide an easy way to ask for help.

A **"Talk to Us"** option should be available throughout the application, allowing families to:

- Describe their situation
- Upload supporting documents
- Request a callback
- Ask general questions

These requests should be directed to the care coordination team for follow-up.

Design Rule

Before adding any feature or information to the Family App, the following question should be considered:

> **Does the family need this information to receive or manage their care?**

If the answer is **no**, the feature belongs in the Staff Portal.

This principle should guide all future design and development decisions.

Design Vision

Riayah Plus should feel like a modern healthcare service, not a hospital information system.

Families should experience a simple, supportive, and guided journey, while clinicians and operational staff have access to the tools and workflows they need to deliver safe, high-quality

care. This separation between the Family App and the Staff Portal is a core design principle and should remain consistent as the platform evolves.

I would add this as a separate section in the PRD because the **Care Coordinator role is a core differentiator of Riayah Plus**. The coordinator is not a receptionist, administrator, or clinician. They are the person who connects the family, clinical team, and operational team to make sure the care plan is created and delivered correctly.

Role Definition – Care Coordinator

Purpose of the Role

The Care Coordinator is the central point of coordination between the patient/family, doctors, allied health professionals, nurses, support workers, and internal operations team.

The coordinator's responsibility is to ensure that every patient receives the right assessment, the right care plan, and the right services according to their needs and goals.

The coordinator does **not replace clinical decision-making**. Clinical decisions remain the responsibility of qualified healthcare professionals.

The coordinator's role is to:

- Understand the patient's and family's needs.
- Coordinate the care journey.
- Ensure information flows between all involved parties.
- Monitor implementation of the care plan.
- Resolve operational barriers.

Difference Between Coordinator and Other Roles

1. Care Coordinator

Main Responsibility:

Owns the care journey.

The coordinator ensures that the patient moves smoothly from initial request → assessment → care plan → service delivery → review.

Main Activities:

Patient Intake Review

When a new patient request is received:

The coordinator reviews:

- Patient information
- Reason for seeking care
- Medical documents
- Existing reports
- Family concerns
- Requested services
- Urgency level

The coordinator decides the next appropriate operational step:

Examples:

- Book consultation
- Request additional documents
- Arrange assessment
- Escalate urgent cases

Consultation Coordination

The coordinator can arrange consultations when needed.

Responsibilities:

- Select appropriate clinician/service
- Check availability
- Schedule appointment
- Communicate appointment details with family
- Ensure required documents are available before consultation

The family can also book consultations directly through the application.

The coordinator provides support when families require assistance.

Family Meeting & Needs Assessment

After reviewing the initial information, the coordinator meets with the family to understand:

- Patient situation
- Family expectations
- Daily challenges

- Personal goals
- Short-term priorities
- Long-term goals
- Preferred care approach
- Family concerns

The coordinator records:

- Family meeting notes
- Patient priorities
- Care preferences
- Environmental factors
- Additional requirements

These inputs become part of the Care Plan creation process.

Care Plan Preparation

The coordinator does not independently create clinical decisions.

Instead, the coordinator combines:

- Family goals and expectations
- Coordinator observations
- Doctor assessments
- Allied health assessments
- Medical reports
- Recommended services

The system can use AI assistance to generate a first draft Care Plan.

Workflow:

Family Meeting Information
+
Clinical Assessments
+
Medical Reports
+
AI Assistance

↓

Draft Care Plan

↓

Coordinator Review

↓

Family Review

↓

Activation

Care Plan Monitoring

After the plan is activated, the coordinator monitors:

- Service delivery
- Appointment completion
- Goal progress
- Family satisfaction
- Missed sessions
- Issues requiring attention

The coordinator ensures the plan is being implemented as intended.

Communication & Coordination

The coordinator acts as the communication bridge.

They can communicate with:

Family

- Explain care plans
- Provide updates
- Answer questions
- Collect feedback

Doctors

- Request reports
- Clarify recommendations
- Arrange follow-ups

Allied Health Professionals

- Coordinate services
- Discuss progress
- Request updates

Nurses and Support Workers

- Clarify tasks
- Follow up on documentation
- Address operational issues

Administration Team

- Resolve scheduling
- Support quotations
- Coordinate payments

The objective is:

> The coordinator makes sure everyone involved in the patient's care is connected and working toward the same goals.

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2. Doctor / Clinical Professional Role

Main Responsibility:

Clinical assessment and decision-making.

Doctors and clinical professionals are responsible for:

- Conducting assessments
- Reviewing medical information
- Creating clinical recommendations
- Documenting findings
- Defining clinical goals
- Providing treatment recommendations

- Reviewing patient progress

They do not manage operational coordination.

They do not:

- Manage staff schedules
- Handle family administration
- Coordinate all services

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3. Allied Health Professional Role

Examples:

- Physiotherapist
- Occupational Therapist
- Speech Therapist
- Psychologist
- Dietitian

Main Responsibility:

Deliver specialized clinical services.

Responsibilities:

- Complete assigned assessments
- Deliver therapy sessions
- Record clinical notes
- Update progress
- Recommend adjustments

They interact with the care plan but do not own the whole patient journey.

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4. Nurse / Support Worker Role

Main Responsibility:

Execute assigned care activities.

Responsibilities:

- Follow assigned care tasks
- Deliver scheduled support
- Record shift/session notes
- Report concerns
- Document incidents

They provide execution data back into the system.

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5. Reception / Administration Role

Main Responsibility:

Administrative operations.

Responsibilities:

- Manage appointments
- Handle payments
- Manage documents
- Support registration
- Assist with scheduling

They do not access unnecessary clinical information.

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6. Family / Patient Role

Main Responsibility:

Participate in the care journey.

Families can:

- Request services

- Book consultations
- Upload documents
- Review care plans
- Approve quotations
- Attend telehealth sessions
- Communicate with the care team
- Track progress

Families cannot:

- Modify clinical records
- Change assessments
- Edit care plans without review

Coordinator Dashboard Requirements

The coordinator dashboard should be designed around managing patient journeys.

New Requests

Shows:

- New patient requests
- Urgency level
- Missing documents
- Required actions

Patient Overview

Shows:

- Patient information
- Family contacts
- Diagnoses
- Reports
- Current care plan
- Active services
- Goals

- Progress

Action Center

Shows tasks requiring attention:

Examples:

- Review new intake
- Schedule assessment
- Request missing report
- Follow up missed session
- Review goal progress
- Contact family

Care Plan Management

Coordinator can:

- Add family goals
- Add consultation notes
- Request AI care plan draft
- Review generated plan and approve
- Monitor implementation

Communication Hub

Single communication area connected to each patient.

Includes:

- Family messages
- Doctor communication
- Therapist communication
- Internal staff communication

All communication should be linked to the patient's Care Episode.

Core Principle

The coordinator is not a booking agent.

The coordinator is the **owner of the patient's care journey**.

Their responsibility is to make sure:

- The patient's needs are understood.
- The right assessments happen.
- The right care plan is created.
- Services are delivered correctly.
- Everyone involved communicates effectively.
- The patient's goals remain the focus throughout the journey.

This role is what differentiates Riayah Plus from a traditional clinic management system. It creates a patient-centred care model where technology supports coordination rather than replacing human relationships.